جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : جليله عبدالحليم مغاورى تغيان
المحافظة : القليوبيه **الحي / المدينة :** بنها **العنوان :** 0
رقم البطاقة : 27906051401761 **رقم التليفون :** 01062466436 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 05-06-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: جليله عبدالحليم مغاورى تغيان
National ID: 27906051401761

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Contrast-enhanced MRI of the brain and orbits**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

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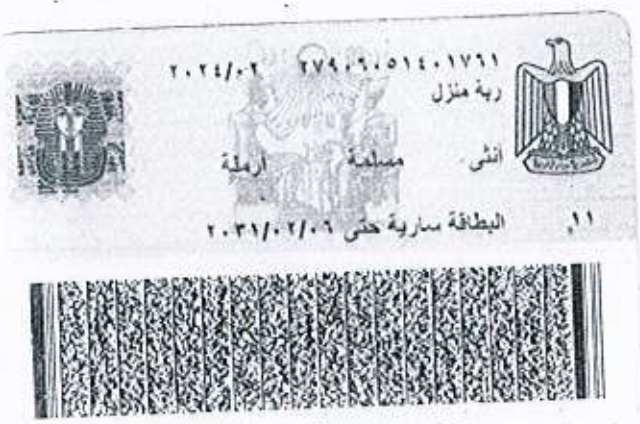
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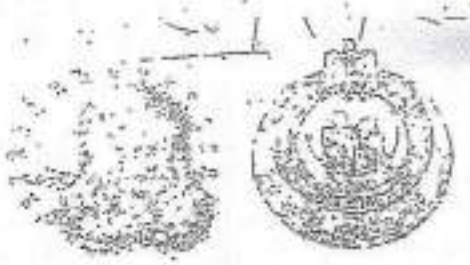
وصل الملف ٢٠٢٧





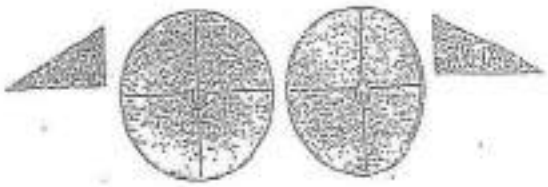
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MDT FORM

Name	
Age	٤٦ .٧٥
MRN	
ID	
Co morbidity	- ٧٤



Clinical Data

Bx: IDC G2	SM:
IHC: ER: ٥/٨ PR: ٥/٨ Her: +3 KI٦٧: —	Mets Workup: Bone scan → Free CT CAP → NAD
initially T_{4c} N_1 M_0	

COMMENT:
a case of left breast. initially was $cT_4N_1M_0$ s/p received 4 AC / 4 Taxotere) Leaf (2) ... s/p 4 Herceptin / Perjeta before surgery the done → simple mastectomy + SLND → ypT0, ypT4b(2), ypMx, Pathological Complete ...

MDT OUTCOME & PLAN

~~For HOT regarding post-operative Anastrozole~~

- Continue Herceptin / perj. (1y)
- Zoladex + A.I + Zometa/6m.
- PORT

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: جليله عبدالحليم مغاوري **MR. :** 4704	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is on adjunctive aromatase inhibitor and zoladex; MRI of the brain demonstrated a small cystic lesion measuring approximately 1 × 1.2 cm. Referral was made for Gamma Knife treatment, and a repeat MRI of the brain along with CT of the chest and abdomen was requested.

The most recent MRI of the brain performed in February 2026 showed no masses, although a prior study in September 2026 had identified a cerebello-pontine angle lesion. The patient is clinically asymptomatic aside from mild headache. Follow-up MRI of the brain is planned in two months to exclude metastatic disease, and a CT of the chest, abdomen, and pelvis was reported as negative for disease; aromasin therapy was continued.

The plan includes administration of aromasin and zoledronic acid (Zometa) next month, continuation of zoladex next month, and repeat brain MRI next month.

The patient will continue aromasin and zoladex with supportive care for three months, with the next follow-up visit scheduled for 11 October 2026.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : حنان عزت شفيق تاودروس

يعتمد ,

Patient Name	: الأستاذه / جليلة عبد الحليم مغاورى	Request No.	: 131112
Patient No.	: 27906051401761	Request Date	: 07/12/2021
Path No.	: 3019/21	Verification Date	: 15/12/2021
Sex	: Female	Financial Category	: اورام دار السلام - اقتصادى
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:		

Pathology Report

Clinical Data:
Left breast cancer. Post chemotherapy

Nature of the speciment:
Left simple mastectomy & sentinel lymph nodes

Gross Examination:
2 specimens were received:
1- Simple mastectomy specimen measured 17x17x5cm, covering skin measured 22x12cm. Nipple and areola were unremarkable grossly. Cut section revealed an UOQ greyish white rubbery tumour bed site measured 3x3x1.5cm, 1cm away from deep margin and 1.5cm from the skin.
2- Sentinel lymph nodes biopsy: A fibrofatty tissue piece measured 3x2cm, containing 2 lymph nodes, up 0.8cm each.

Microscopic examination:
- Histological type: No remnants of tumour tissue.
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Lymph node involvement: All sentinel lymph nodes were free (0/2)
- Resection Margins: Free deep margin
- Histologic response to therapy: Pathological complete response.
- Additional pathologic findings: Free nipple and areola
- Frozen section: Gross: As described above.

Microcscopic: All sentinel lymph nodes were free (0/2)

Diagnosis:
Left simple mastectomy & sentinel lymph nodes:
No remnants of tumour tissue.
Pathological complete response.
Free nipple, areola and deep margin

Branch No. (4) - First Floor Building No. 941 - Nile Corniche - Sakr El Quderaa - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / جليلة عبد الحليم مغاورى	Request No.	: 131112
Patient No.	: 27906051401761	Request Date	: 07/12/2021
Path No.	: 3019/21	Verification Date	: 15/12/2021
Sex	: Female	Financial Category	: اورام دار السلام - اقتصادى
Age	: 40 Y	Department	: Out Patient
Referreing Doc :			

Pathology Report

All sentinel lymph nodes were free (0/2)
Stage: ypT0, ypN0, ypMx

Prof. Dr. Amal Hareedy
Dr. Abdulrahman Ibrahim

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مستشفى أورام دار السلام
قسم الباثولوجى

Pathology Report

Name: جليله عبدالحليم مغاورى تفيان Age: 40years.

Clinical data: Left breast mass.

Specimen type: Tru cut biopsy.

Date (Received): 16/3/2021 Date(immuno): 11/4/2021

Lab. Number: 489/21 Hospital Number: مبادرة

Gross Description:

Three greyish white cores biopsies, measuring 1cm long each, totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2
ER clone (1E2).
PR clone (SP1).
HER-2 CLONE (4B5).

Microscopic Examination:

histopathological examination of sections prepared from the true cut biopsy received reveals a malignant epithelial growth. The tumor made up of sheets and small groups of moeratly atypical ductal epithelial cells, having pleomorphic nuclei, surrounded by desmoplastic stroma

- Microscopic examination of ER stained slide show moderate nuclear stain in>30% of the tumor cells.
- Microscopic examination of PR stained slide show moderate nuclear stain in 50% of the tumor cells.
- Microscopic examination of HER2 stained slide show strong staining in > 10% of tumor cells with complete membranous staining.

Diagnosis:

left breast mass, tru cut biopsy:

- **Invasive duct carcinoma grade II**
- **ER: Positive (5/8).**
- **PR: Positive (6/8).**
- **HER-2: positive (score 3+).**

Prof.Dr Gamal Abdel-Aty
Dr. Rana Fathy

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مستشفى أورام دار السلام
قسم الباثولوجى

Pathology Report

Name: جليله عبدالحليم مفاورى تغيان Age: 40years.

Clinical data: Left breast mass.

Specimen type: Tru cut biopsy.

Date (Received): 16/3/2021 Date(reported): 22/3/2021

Lab. Number: 489/21 Hospital Number: مبادرة

Gross Description:

Three greyish white cores biopsies, measuring 1cm long each, totally submitted.

Microscopic examination:

histopathological examination of sections prepared from the core biopsy received reveals; an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II].

Diagnosis:

Left breast mass, core biopsy:

- ***Invasive duct carcinoma grade II.***

Prof. Dr. Amal Hareedy.
Dr. Mona Mohamed.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مستشفيات جامعة بنهـــا
BENHA UNIVERSITY HOSPITALS

تقـــرير أشعـــة

Patient Name English: Galilah Abdalhalim Magawery Taghian	Patient ID: 135623
Age: 46 Year(s), 3 Month(s)	Modality Name: MR
Study Date: 29/09/2025 09:15:00 AM	Sex: Female

MRI OF THE BRAIN

❖ TECHNIQUE:

- Axial T1WI.
- Axial T2WI.
- Axial FLAIR.
- Coronal T2 and sagittal T1WI.
- Postcontrast axial, coronal & sagittal T1WIs.

❖ CLINICAL BACKGROUND:

- Know case of breast cancer with extra-axial S.O.L , presented for follow up, yet no available previous films.

❖ FINDINGS:

- Right frontal dural based extra-axial space occupying lesion measuring about 12X4X10 mm at its maximum axial and CC diameters respectively displaying isointense T1WI & T2WI signal and hyperintense T2WI signal with homogenous postcontrast enhancement and enhancing dural tail, showing peripheral rim of signal drop-out at SWIs.
- Dilated ventricular system with widened basal cisterns, sylvian fissures cortical sulci and other extra-axial CSF spaces.
- No shift of midline structures.
- No cerebral, cerebellar or brain stem areas of abnormal MRI signal intensity.
- No intra axial masses or extra axial collections.
- **Right cerebellopontine angle cystic lesion of CSF signal measuring about 1.8X0.9X1.8 cm at its maximum axial and CC diameters respectively.**
- **Partially empty Sella, prominent mickle's caves with low lying cerebellar tonsils about 4 mm below the foramen magnum, idiopathic intracranial hypertension is suggested for clinical assessment.**

M.R.I
[[DOCTYPE:MRI]]

Name	Mrs. Galila Abdalhaleem Maghawry Tagyin		
Ref. By Prof. Dr.	Center Awram Mit Ghamr	Date	23/9/2025
ID	6625013581	Acc. No.	295013663

^{18}F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of managed left breast malignancy, IDC (pathology report, 03.2021)

Total body PET scan was done 40 minutes after IV administration of 300 MBq of 18F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROI's & the normal threshold would be the normal blood pool liver SUV max< 2

Reported By:

Dr. Mm Abd El Razek

Approved By:

Prof. Dr.

Essam El-Shrekh

E. checked by: Dr.

Typed by: SH. Mo

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain, with no definite intraxial metabolically active SOLs.

(N.B; not all brain metastatic lesions could be detected on 18-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).

- Normal naso-/oro-pharynx, larynx, thyroid and salivary glands.
- No enlarged FDG hypermetabolic cervical L.Ns seen.

Torso:

Thorax:

- **Left mastectomy with no focal operative bed active masses.**
- **No enlarged FDG hypermetabolic mammary related lymph nodes.**
- **For mammographic correlation.**
- **Left pulmonary post irradiation changes.**
- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.

Abdomen & Pelvis:

- Enlarged liver with no FDG hypermetabolic focal lesions. For dynamic MRI assessment if warranted.
- No biliary tree dilatation.
- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses), adrenals and pelvic organs.
- No enlarged FDG hypermetabolic abdominal or pelvic L.Ns seen.
- Normal physiological biodistribution over the bowel & urinary tracts.

PET Scan
[[DOCTYPE:PET_SCAN]]

ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Galila Abd Al Haleem Magawery	Date : 14/08/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 46 Year(s)
Medical ID: 824/2021	Pat. ID: 30655

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Compared to previous contrast-enhanced study dated 08/08/2024, the current study shows:

I Chest:

Stationary course regarding:

- Left MRM with no operative bed mass lesions.
- Still noted left upper lung lobe subpleural reticulations, post RTH.
- Stable few tiny left pulmonary nodules, the largest seen at left lower lung lobe anterior basal segment measuring about 4 mm some are calcified
- Still noted bilateral, predominantly left, apical reticulations, likely of old granulomatous affection.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions (on non-contrast basis) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- **Newly noted** few peritoneal nodules at right lumbar region measuring about 7 x7 mm.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Scanned skeleton show:

- Still noted multi-level vertebral lytic lesions with vertical striations likely hemangiomas.
- Still noted right superior pubic ramus sclerotic lesion.

Opinion:

- **Newly noted few peritoneal nodules**
- **Rather Stationary course of the rest of the study .**

DR/Reham Khedr ,,MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Oncology Center
مركز الأورام

NASSER INSTITUTE
خطابات أورام
ONCOLOGY CENTER
NUCLEAR MEDICINE UNIT

وزارة الصحة
أمانة المراكز الطبية التخصصية
مستشفى ناصر
Nasser Institute Hospital
معهد ناصر
مركز الأورام
وحدة الطب النووي

Patient Name: Galila AbdElhalim Maghawry.. **Date:** 25/5/2022

Tc-99m MDP bone scan.

Desired information : skeletal surveying to detect multiple lesions.

Technique:

Delayed Anterior and posterior whole body scan 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Delayed Static Phase:-

- No definite hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- . Degenertaive changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are normally seen

Conclusion:

- ***No definite evidence of metastatic osseous deposits.***

Thanks for reference

Nuclear medicine Unit

Dr.Huda Fathy , MD ***Dr.Sherine Mahdy,MD***

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فاكس : ٢٢٠٢٩٢٣٦
...Corniche El Nile - Aghakhan - Shoubra - Cairo Tel.: 22039...

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Women's Health Outreach Program

Name: Galil Abd el halim meghawry
Date: 31 08.2021

Contrast Enhanced Full field Digital Mammography

Clinical summary: left breast IDC grade II.

Technique
Pre & post contrast Mediolateral oblique and CC views
Targeted High resolution ultrasound

Detailed Findings

Left breast

- Dense fibroglandular pattern (ACR C)
- 1- Ill defined irregular upper midline malignant mass associated with microcalifications.
- 2- UOQ clusters of microcalcifications..
- **Contrast enhanced images demonstrated:**
- 1-the previously described mass showed heterogeneous intense contrast uptake, located at 12:00 position about 7.5 cm from the nipple and 2 cm from the underlying muscle.
- Few small enhancing nodules are seen surrounding the index mass.
- index mass + enhancing nodules = 6 x 5 x 4.7 cm.
- 2-ill defined faint non mass enhancement is seen located at 2:00 about 8 cm from the nipple, measure 4.8 x 2.7 cm in diameter.
- Normal skin/nipple/areolar complex.
- Pathological axillary LNs on targeted US.

Right breast

- Dense fibroglandular pattern (ACR C)
- no dominent mass lesion identified.
- **Contrast enhanced images showed:** no pathological mass or non mass enhancement.
- no signs of malignancy.
- Preserved axillary LNs on targeted US .

Conclusion:

-Left breast: (BIRADS 6 = Pathology proved malignant). upper midline malignant masses and UOQ non mass enhancement.
-Right breast: (BIRADS 1= negative). normal study

Best regards,

Mammogram
[[DOCTYPE:MAMMOGRAM]]